

OCD: Patient Information Leaflet

Intrusive, unwanted thoughts (obsessions) that cause intense anxiety, temporarily relieved by repetitive behaviors or mental acts (compulsions).

What is Obsessive-Compulsive Disorder (OCD)?

Obsessive-Compulsive Disorder involves obsessions — recurrent, intrusive, unwanted thoughts, images, or urges that cause significant anxiety — and/or compulsions, repetitive behaviors or mental acts performed to reduce that anxiety or prevent a feared outcome. Common themes include contamination and cleaning, symmetry and ordering, fears of causing harm, and unwanted taboo or aggressive thoughts. Compulsions provide only brief relief, which paradoxically strengthens the cycle, making the obsessions return more strongly over time.

Common symptoms

- Contamination obsessions (fear of germs, dirt, illness) with washing or cleaning compulsions
- Fear of causing harm to oneself or others, with checking compulsions (stoves, locks, re-reading)
- Need for symmetry or exactness, with ordering or arranging compulsions
- Unwanted taboo, violent, or sexual intrusive thoughts, often causing intense shame
- Mental compulsions such as silent counting, praying, or repeating phrases
- Excessive reassurance-seeking from others
- Significant time (often hours daily) spent on obsessions or compulsions

What causes it

OCD has a strong genetic component, with heritability estimates around 40–50%. It is understood as a disorder of the brain's error-detection and habit-formation circuitry,

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infection (PANDAS/PANS) points to an autoimmune-mediated mechanism, though this remains a specific and less common presentation.

Treatment options

Exposure and Response Prevention (ERP), a specialized form of CBT, is the gold-standard psychotherapy for OCD and produces durable improvement for most patients. SSRIs are first-line medication but are typically needed at higher doses than used for depression. Combination treatment (ERP plus an SSRI) is generally recommended for moderate-to-severe OCD.

Self-help tips

- Practice resisting compulsions gradually rather than all at once, following an ERP hierarchy
- Avoid seeking repeated reassurance from family or online sources, which reinforces the OCD cycle
- Track obsession/compulsion time to notice gradual improvement
- Maintain regular sleep and stress-management routines, since stress intensifies symptoms
- Involve family in understanding ERP so home life doesn't inadvertently accommodate rituals

Prognosis

Without treatment, OCD tends to follow a chronic, waxing-and-waning course. With ERP and/or medication, the majority of people achieve substantial symptom reduction, and many achieve full remission, though a subset with severe or treatment-resistant OCD may need combination treatment or, rarely, more advanced interventions in specialist settings.

When to seek urgent help

Seek urgent psychiatric care if OCD is causing an inability to work, attend

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damage from excessive washing), or if obsessions are accompanied by thoughts of self-harm or suicide.

This material is for patient and family education only and does not replace individualized medical advice. For a personal diagnosis or treatment plan, consult Dr. Kushal Kharel or a qualified mental health professional.

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